What happens after people show up?

How beneficiary and staff feedback can show what worked, where people experienced difficulty, and what a programme could improve next.

PRACTICE DATA | 500 BENEFICIARIES | 50 FIELD STAFF | 5 STATES

67.4%
said something made it difficult to come

46 min
was the average waiting time

29%
wanted shorter waiting times

3 in 4
heard about outreach through community health workers, health facility or community leader

Why I looked beyond attendance

Outreach reports usually tell us how many people came. That matters, but it doesn't tell us whether getting there was easy, what the visit felt like, or what should change next time.

I built this analysis to show what becomes possible when we ask two simple questions: **What did the people who came experience?** And: **What did the staff who served them experience?**

The data is synthetic and is used only to demonstrate an analytical approach.

What the beneficiary data says

People came, but many still faced difficulties getting there.

Two out of every three beneficiaries reported a main barrier before attending. The most common were lack of information, distance or transport cost, and work or household responsibilities.

What this means: Attendance does not automatically mean access was easy. Programmes also need to reduce the barriers that make access difficult.

The longer people waited, the less satisfied they were.

The average wait was 46 minutes. One in five people waited more than an hour and rated their visit 2.8 out of 5, compared with 3.8 for those who waited 30 minutes or less.

The data does not show that waiting caused lower satisfaction, but longer waits were generally linked with lower satisfaction.

What this means: Tracking waiting time can help programme teams spot service problems and see whether changes improve the experience.



People also told us what they want changed.

Shorter waiting times were the most common request (29%), followed by more outreach days (21%), clinics closer to home (18%), and more information before outreach (17%).

What this means: Beneficiary feedback can move the conversation from “what happened?” to “what should we try next?”

Putting the two sides together

What beneficiaries experience and what staff see are not always the same

The staff data adds an operational view. Transport and logistics were the most common specific challenge reported by staff (18%), followed by long waiting time (16%), documentation workload (14%), and insufficient staff (12%).

There is agreement on the information.

Staff rated their community awareness work lowest in Katsina and Lagos. Those are also the two states where people most often named lack of information as their biggest barrier to coming: about 1 in 4 people, compared with roughly 1 in 8 to 1 in 6 elsewhere.

This does not prove that one problem caused the other. It shows that staff and beneficiaries point to the same issue.

There are also useful differences.

Kano had the highest average waiting time among the five states, at about 50 minutes, yet only one of its 14 staff records named long waiting time as the main operational challenge.

Why that matters: Staff may not experience a queue in the same way beneficiaries do. Comparing both perspectives can reveal issues that one dataset alone would miss.

What a programme team could do next

Fix the queue first. Review registration and queue flow at sites with consistently long waits. Fagge, in Kano, for example, recorded an average wait of about 61 minutes in this dataset.

Tell people earlier. Where information gaps are common, review when outreach dates are announced and whether community channels are being used effectively.

Look at where outreach happens. Where distance or transport is a recurring problem, check whether outreach locations and schedules are making access difficult.

Measure the experience. Record waiting times consistently so teams can see where delays occur and whether changes actually help.

Connect beneficiary and staff records. Linking the two datasets by outreach location and date would make it easier to see where staff and beneficiaries agree, and where they do not.

What I bring

I combine health context with practical data skills. My training in microbiology gives me a health-science grounding, while my experience with Excel, SQL, Python and Power BI helps me clean, analyse and communicate data.

My focus is decision-first. I want analysis to answer a programme question clearly, explain what the evidence does and does not say, and make the next decision easier.

The goal is not simply another report. It is to turn field activity into information that programme teams can understand, question, and use.

I would welcome a short conversation about where field data support could help your team.

COLLECT → CHECK → UNDERSTAND → COMPARE → ACT

Sharon Imaledo · Data Analyst · sharonimaledo@gmail.com · LinkedIn: linkedin.com/in/sharon-imaledo-767667213